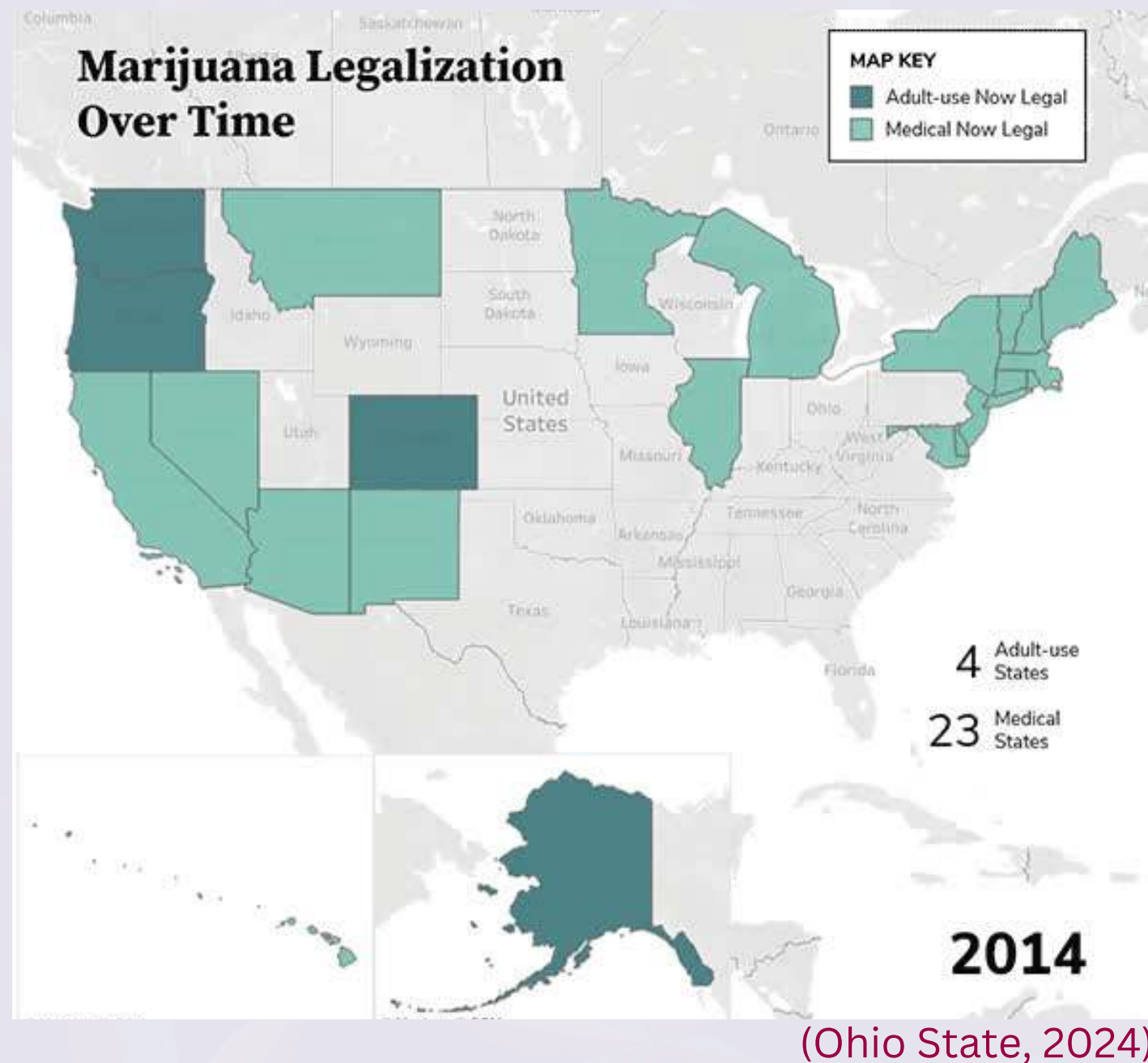




Joint Realities
**Patterns of Cannabis Use
Among Sexual Minority Adults**

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Cannabis Use in Context: Risk, Identity, and Surveillance Gaps

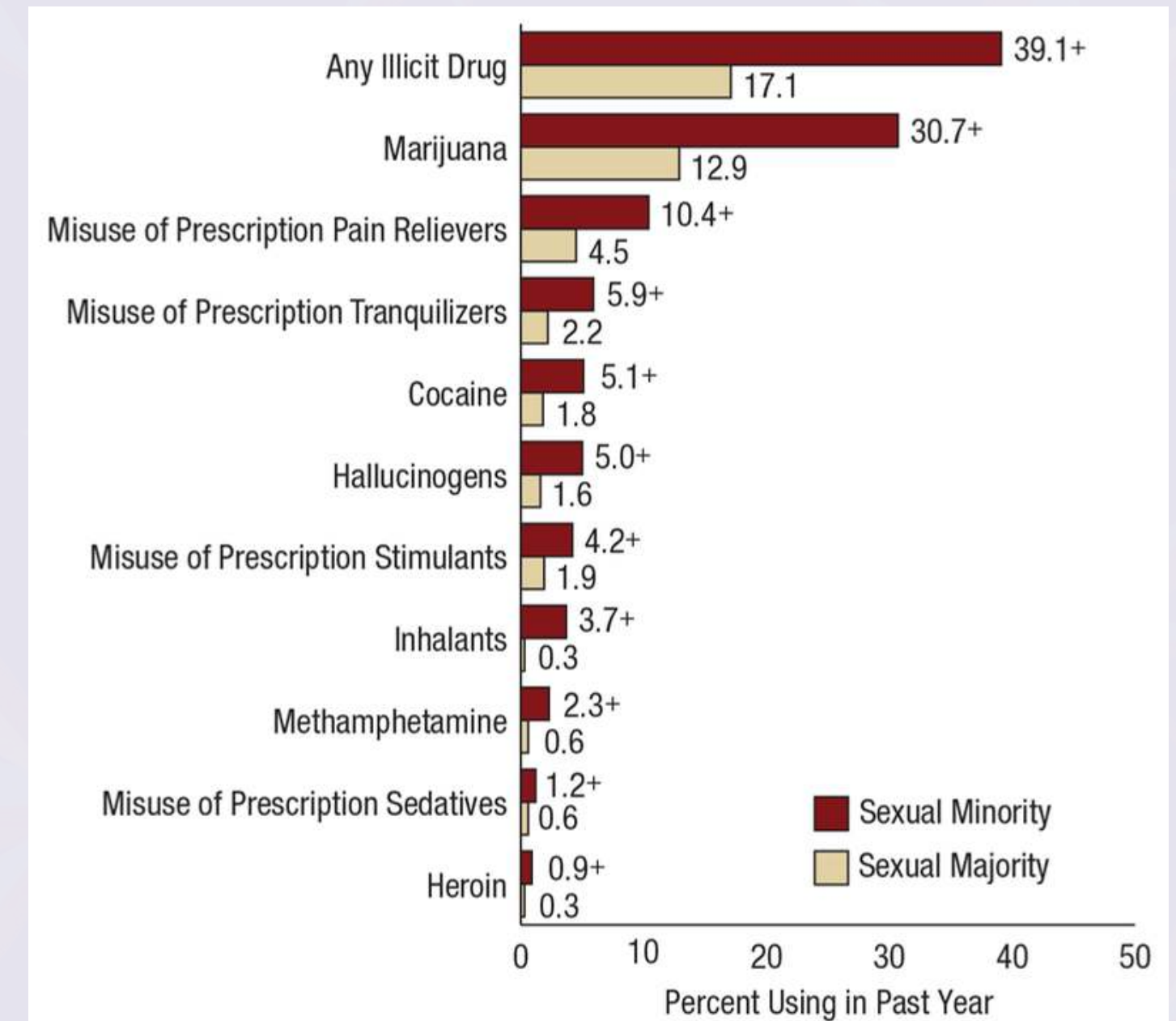


- Over the past decade, cannabis use has grown tremendously across the United States.
 - Nearly **1 in 5** adults report past-month cannabis use (NSDUH, 2022).
 - Now the most commonly used substance outside of alcohol
- State-level legalization has expanded rapidly, but cannabis remains federally illegal as a **Schedule I substance**.
 - Public health research on cannabis use remains limited, partly due to research restrictions on Schedule I drugs.
- National surveys (e.g., NSDUH, BRFSS, NHANES) track overall trends, but often miss nuances in frequency, modality, and patterns of use.

LGBTQ+ Health in Context:

Mental Health, Substance Use, and Systemic Barriers

- LGBTQ+ individuals experience significantly higher rates of mental health conditions compared to heterosexual and cisgender peers
 - Approximately **twice as likely** to experience depression, anxiety, suicidality, and substance use disorders
- High overlap exists between LGBTQ+ populations and conditions frequently qualifying for medical cannabis use (e.g., anxiety, PTSD, chronic pain)
- Major gaps in national data collection limit our understanding of cannabis use patterns in LGBTQ+ communities



(SAMHSA, 2015)

Introducing the Current Study:

Data Source, Sample, and Key Variables

The Behavioral Risk Factor Surveillance System (BRFSS) is the largest ongoing health survey system in the U.S., collecting annual state-level data on health behaviors and outcomes.

Across 2022 and 2023, **eight states** included the cannabis module: Connecticut, Delaware, Illinois, Indiana, Montana, Nevada, New Mexico, and Virginia.

Key variables used in this project:



Cannabis Module: Past-month cannabis use, frequency of use, and methods of consumption.



SOGI Module: Sexual orientation and gender identity.

- disaggregated by sex and referred to as “sexual orientation-sex group”



Sociodemographics: Age, race/ethnicity, education, income, employment, marital status, self-rated health, and insurance coverage.

Aims

1

Estimate the **prevalence** of past-month cannabis use stratified by sexual orientation–sex group

2

Assess the **frequency** of cannabis use among past-month users, comparing rates across sexual orientation–sex groups.

3

Evaluate cannabis consumption **modality** and **multi-modal** useage across sexual orientation–sex groups among past-month users

Aim 1:

Prevalence of Past-month Cannabis Use

- Prevalence varied significantly across sexual orientation–sex groups among both women and men.

■ Bisexual women	(39.7%)
■ Bisexual men	(32.5%)
■ Lesbian women	(25.5%)
■ Gay men	(29.6%)
■ Straight men	(17.2%).
■ Straight women	(11.5%).

- Bisexual women had **significantly greater odds** of cannabis use compared to heterosexual women (AOR = 2.49, $p < .001$).
- Both bisexual (AOR = 1.57) and gay men (AOR = 1.64) had **significantly greater odds** of use than heterosexual men.

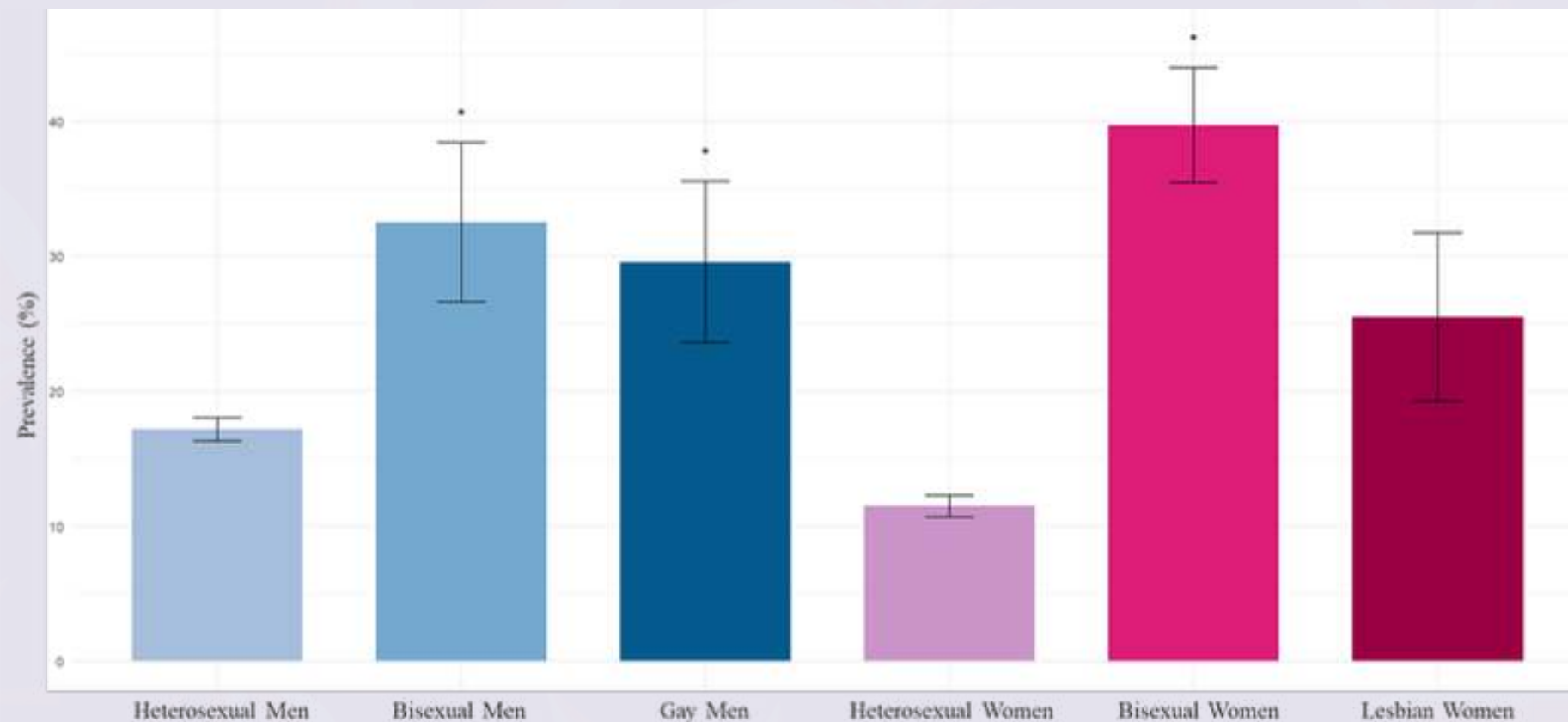


Figure 1. Weighted Prevalence of Past-Month Cannabis Use by Sexual Orientation–Sex Group (BRFSS 2022–2023)

Note. Bars represent weighted prevalence; error bars indicate 95% confidence intervals. Asterisks (*) indicate statistically significant differences ($p < 0.05$) in prevalence compared to the heterosexual reference group within each sex category.

Aim 2: Frequency of Cannabis Use

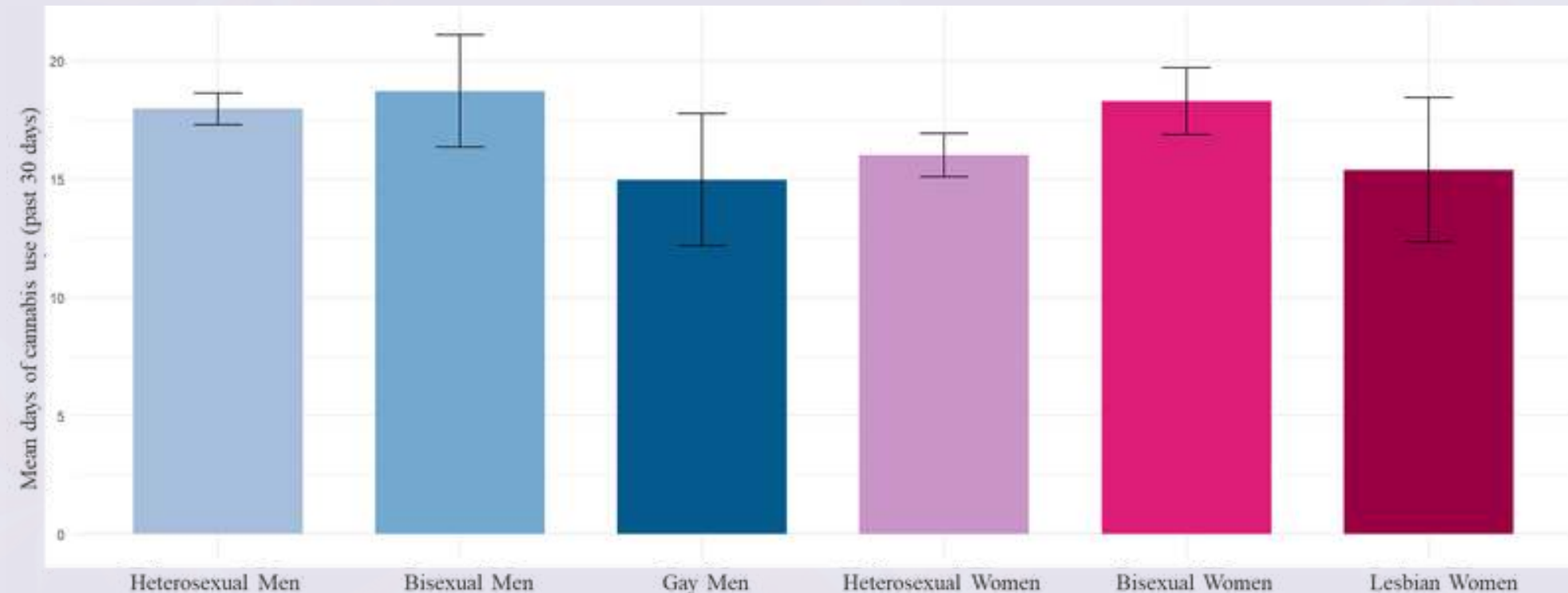


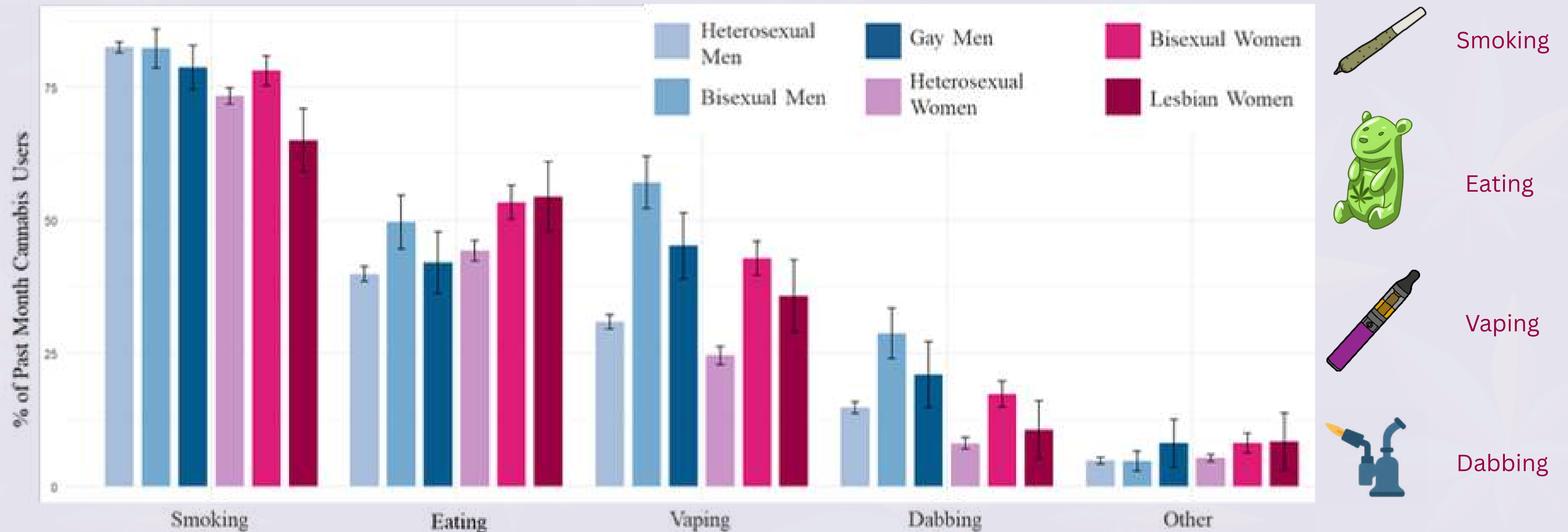
Figure 2. Weighted Mean Days of Cannabis Use in the Past Month by Sexual Orientation–Sex Group (BRFSS 2022–2023).
Note. Error bars represent 95% confidence intervals. Estimates are stratified by sex assigned at birth and based on survey-weighted linear regression models.

Mean days of cannabis use varied across sexual orientation–sex groups

Bisexual men	18.7
Bisexual women	18.3
Heterosexual men	18.0
Heterosexual women	16.0
Lesbian women	15.4
Gay men	15.0

- Bisexual women reported **significantly more frequent cannabis use** compared to heterosexual women (IRR = 1.14, $p = .044$).
- No significant differences in use frequency by sexual orientation among men after adjustment.

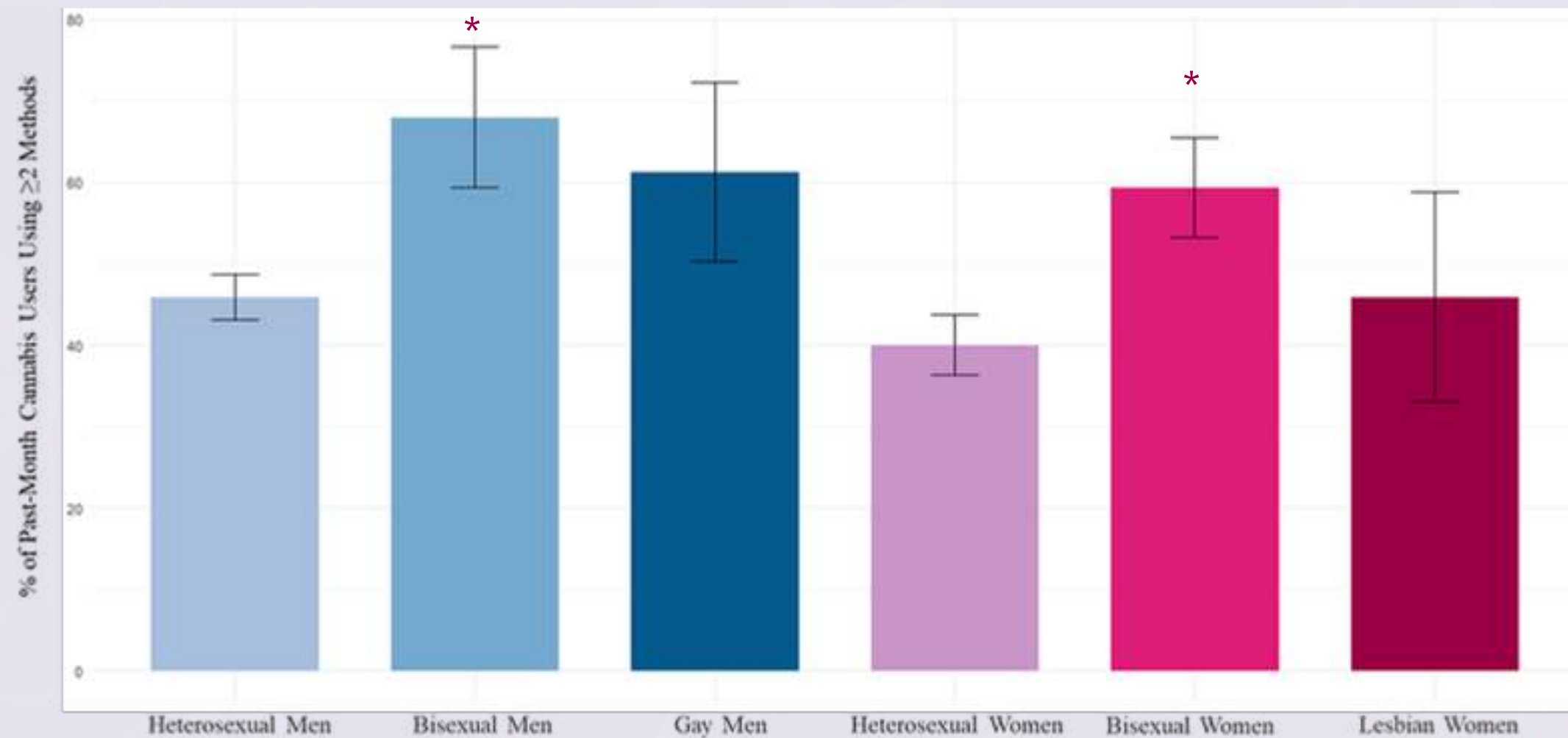
Aim 3a: Cannabis Use Modality



- The most common method of cannabis use was **smoking** across all sexual orientation–sex groups
- **Sexual Minority individuals reported higher prevalence of vaping, edibles, and dabbing** compared to their heterosexual peers.
- Gay men reported higher “other” methods

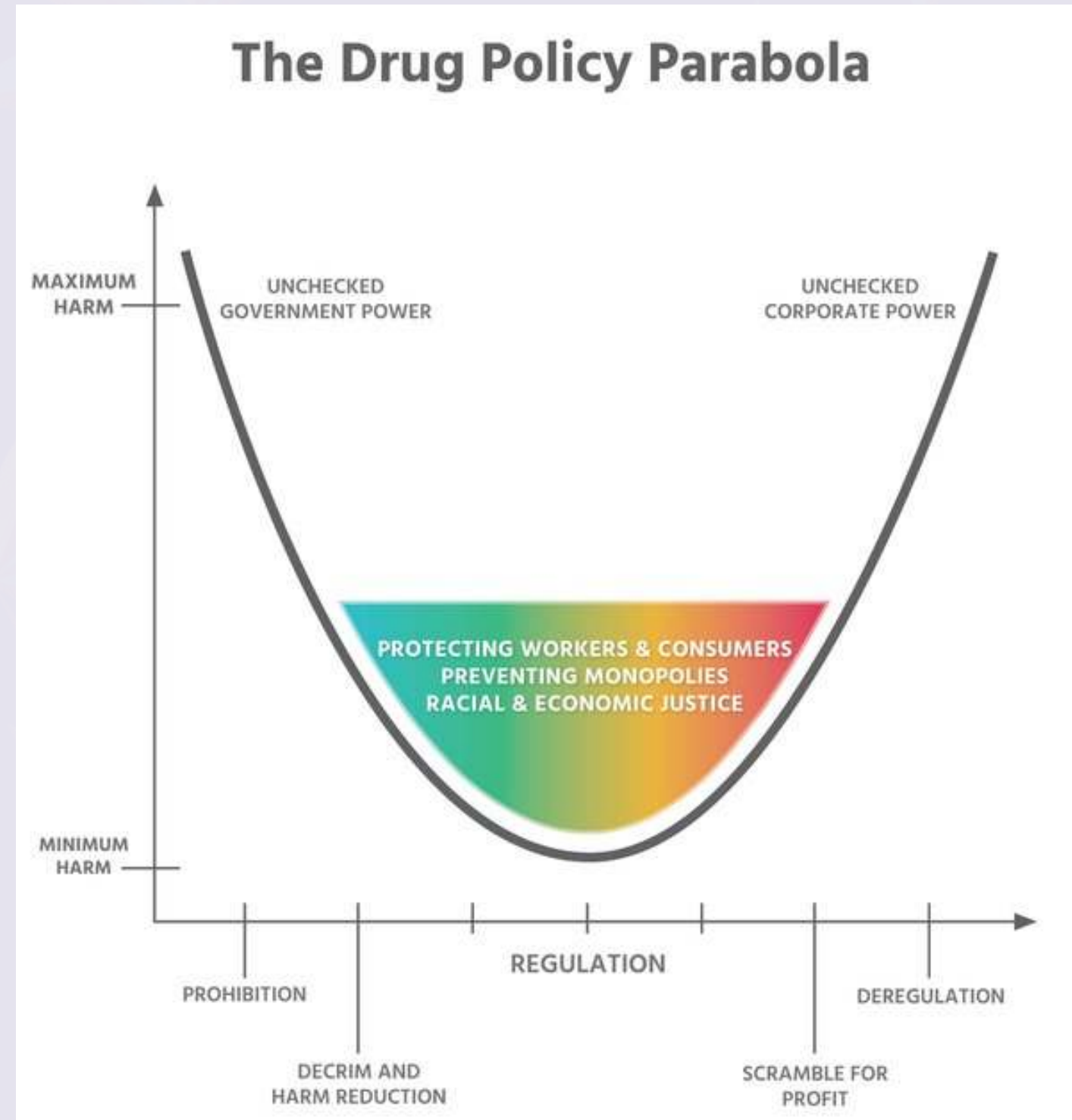
Aim 3b: Multi-Modal Cannabis Use

Multimodal consumption was defined as reporting use of two or more methods within the past 30 days.

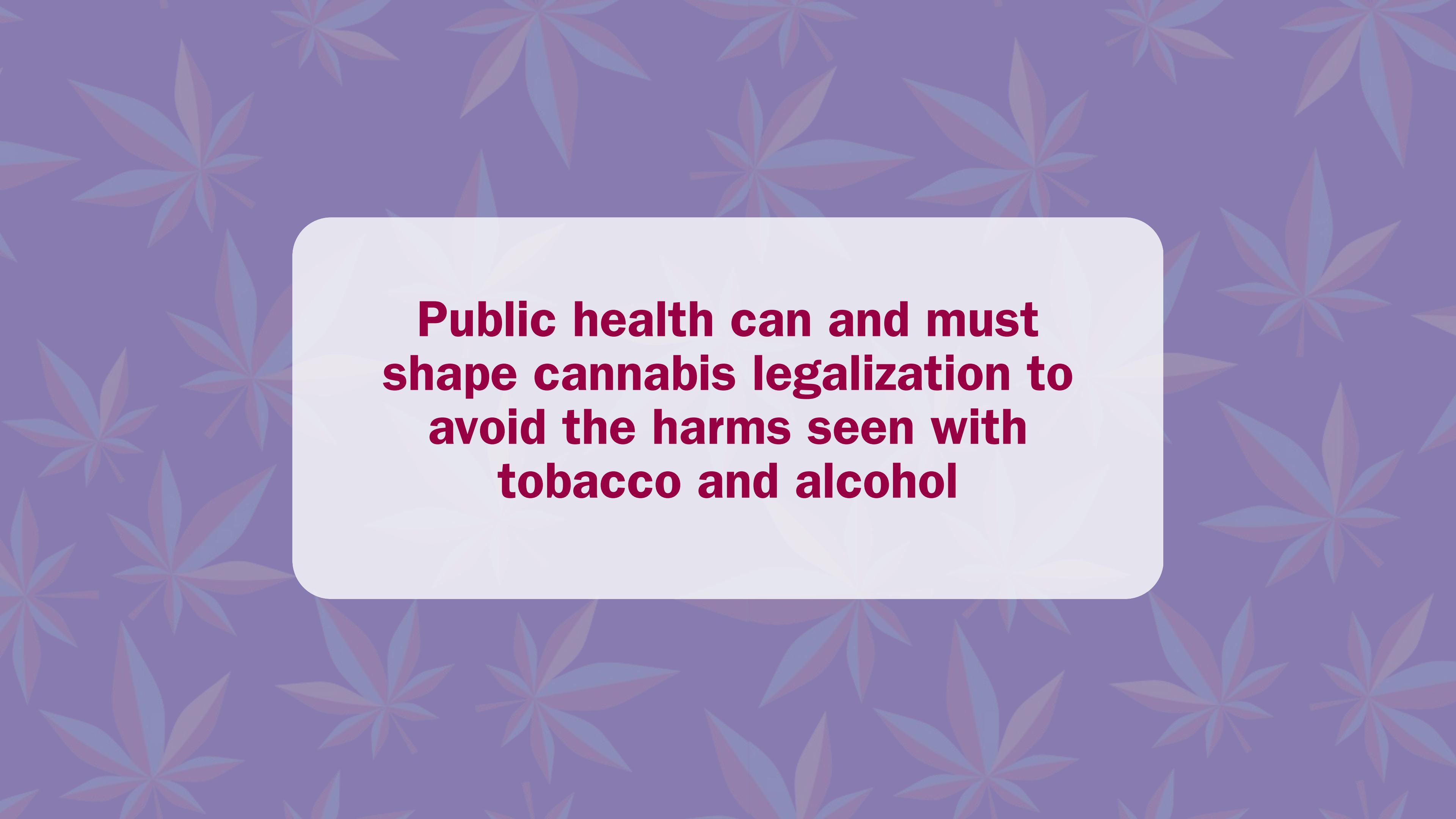


- Bisexual women had **significantly higher odds of multimodal use** compared to heterosexual women (AOR = 1.88, $p = .002$)
- Bisexual men had **significantly higher odds of multimodal use** compared to heterosexual men (AOR = 2.30, $p = .003$)
- No significant differences in multimodal use between lesbian and heterosexual women or between gay and heterosexual men

Public Health Implications



- Cannabis use is changing, especially in LGBTQ+ communities — **public health must keep pace**
- Substance use is a constant; **prohibition alone doesn't protect health.**
- Both medical (e.g., anxiety, depression) and recreational use matter, especially for LGBTQ+ populations.
- Different methods (smoking, edibles) carry distinct risks, needing clear education.



**Public health can and must
shape cannabis legalization to
avoid the harms seen with
tobacco and alcohol**



Thank You!
Questions?

Additional Graphics From:

Sexual Orientation and Estimates of Adult Substance Use and Mental Health:
Results from the 2015 National Survey on Drug Use and Health

<https://www.samhsa.gov/data/sites/default/files/NSDUH-SexualOrientation-2015/NSDUH-SexualOrientation-2015/NSDUH-SexualOrientation-2015.htm>

and

Ohio State Moritz Drug and Policy Center

https://moritzlaw.osu.edu/faculty-and-research/drug-enforcement-and-policy-center/research-and-grants/drugs-on-the-ballot?utm_campaign=law_marketing-activity_fy25